

TECHNICAL DATA SHEET FOR MALARIA MANAGEMENT

Mandatory test for any clinical suspicion of malaria: Thick Gout (TG) or Rapid Diagnostic Test (RDT)

I. TREATMENT OF UNCOMPLICATED MALARIA

Diagnosis based on : Fever, Chills, Generalized asthenia, Headache, Sweating, Joint pain, Vomiting, Diarrhea especially in children, Limping, Anorexia

1. FIRST-LINE TREATMENT

Artemether 20mg+Lumefantrine 120mg : Number of tablets per dose						
Weight	J1			J2		J3
	0h	8h	20h	32h	44h	56h
5à14kg : Blister of 6	1	1	1	1	1	1
15 to24kg : Blister of 12	2	2	2	2	2	2
25 to34kg : Blister of 18	3	3	3	3	3	3
35 kg and over: Blister of 24	4	4	4	4	4	4

Management of fever with Paracetamol 500mg (paracetamol 15 mg/kg every 4 hours p. 3 to 5 days< 3 years: ¼ cp 3 - 5 years: ½ cp 6 - 9 years: 1 cp> 10 - 14 years: 1 ½ cps> 14 years 2 cps+ Feeding and rehydration

A few useful tips

- ✓ Systematic weight measurement for all patients
- ✓ The first dose should be administered under the direct observation of the provider.
- ✓ In the event of vomiting within 30 minutes of taking the drug, the dose must be repeated.
- ✓ If they occur after 30 minutes, the drug is considered to have been fully absorbed, so there's no need to repeat the dose.
- ✓ Ensure that patients have a clear understanding of AL dosing intervals
- ✓ For nursing mothers: no contraindications
- ✓ If the patient's general condition deteriorates 1-2 days after the end of the prescribed treatment, consult the health facility again.
- ✓ Fever management: Moist wrap with lukewarm water or ventilation to lower temperature

2. SECOND-LINE TREATMENT

Confirmation of relapse by thick drop or smear necessary. An RDT can be , as it can remain positive for over a month.

- Dihydro artemisinin+piperazine cp (DHA-PPQ) 20mg+160mg dispersible cp
3;40mg+320mg blister pack Blister of 3;40mg+320mg Blister of
6 ;40mg+320mg Blister of
9 ;40mg+320mg Blister of 12.

The recommended dose is 6 doses of 1 to 3 tablets, depending on the patient's weight.

- Quinine is available in coated tablets of 100mg, 300mg and 500mg, while clindamycin is available in 150mg and 300mg capsules.

a. TREATMENTS APPLICABLE TO SPECIFIC POPULATIONS

- Pregnant women: Quinine+clindamycin combination in the first trimester, and Artemether-Lumefantrine in the 2nd and 3rd trimesters.
- Infants: infants under 2 months of age or weighing less than 5 kg are considered severe cases and will be managed as severe malaria: Artesunate IV injection at a dose of 3mg/kg for 7 days
- PLHIV on ARV and immunosuppressive therapy will be treated with Artemether-Lumefantrine.
- Patients suffering from severe malnutrition with uncomplicated malaria will be treated at Artemether-Lumefantrine as treatment.

II. TREATMENT OF SEVERE MALARIA

Diagnosis based on : Prostration, Uncontrollable vomiting, Coma, Respiratory distress, Circulatory collapse (shock), Renal failure, Hemoglobinuria, Jaundice, Pulmonary edema

Refer urgently after initiating specific antimalarial treatment with IV/IM Artesunate, or failing that, parenteral infusion of Quinine 5 or 10% Glucose.

Artesunate Injection dosage

- ✓ Dosage in children under 20kg: 3mg/kg
- ✓ Dosage in adults and children over 20kg: 2.4mg/kg

b. TREATMENTS APPLICABLE TO SPECIFIC POPULATIONS

- Treatment of severe malaria in pregnant women: Quinine injection in the 1st trimester and Artesunate injection in the 2nd and 3rd trimesters.
- Treatment of severe malaria in the malnourished: Artesunate injection.